

1. Administrative & Study Identification

Full Study Title: A Phase III Randomized, Double-Blind, Placebo-Controlled, Multicenter Study Evaluating the Efficacy and Safety of Giredestrant Combined With Palbociclib Compared With Letrozole Combined With Palbociclib in Participants With Estrogen Receptor-Positive, HER2-Negative Locally Advanced or Metastatic Breast Cancer **Short Title/Acronym:** persevERA Breast Cancer **Protocol Number:** B041843 **NCT Number:** NCT04546009 **Sponsor Name:** Hoffmann-La Roche **Trial Status:** Active but not currently recruiting (ACTIVE_NOT_RECRUITING) **Investigational Product:** Giredestrant (GDC-9545) + Palbociclib **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead [Insert Contact Information]

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of giredestrant combined with palbociclib compared with letrozole combined with palbociclib as first-line treatment for ER-positive, HER2-negative locally advanced or metastatic breast cancer, measured by investigator-assessed Progression-Free Survival (PFS). **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), Duration of Response (DOR), Clinical Benefit Rate (CBR), Quality of Life (QoL), pharmacokinetics (PK), and safety profiles of the treatment regimens.

2.2 Background & Rationale To address the need for more effective endocrine-based first-line therapies for patients with estrogen receptor (ER)-positive, HER2-negative locally advanced or metastatic breast cancer. This study assesses whether the novel oral selective estrogen receptor degrader (SERD) giredestrant, combined with the CDK4/6 inhibitor palbociclib, can provide superior progression-free survival over the current standard-of-care aromatase inhibitor (letrozole) combined with palbociclib in this patient population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Letrozole) / Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized 1:1

3.2 Planned Enrollment & Duration Target \$N\$: 992 enrolled participants **Number of Sites:** ~256 global locations **Estimated**

4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria** 1. Postmenopausal or pre/peri-menopausal women (if treated with LHRH agonist therapy for the duration of study treatment) or men. 2. Locally advanced or metastatic adenocarcinoma of the breast not amenable to curative intent therapy. 3. Documented Estrogen Receptor (ER)-positive and HER2-negative tumor status. 4. No prior systemic anti-cancer therapy for metastatic disease (1L setting). 5. Measurable disease per RECIST v1.1. 6. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1.
- 4.2 Exclusion Criteria** 1. Disease recurrence within 12 months of prior treatment with an aromatase inhibitor (AI) or a CDK4/6 inhibitor in the adjuvant/neoadjuvant setting. 2. Prior treatment with a Selective Estrogen Receptor Degradar (SERD) or fulvestrant. 3. Active, symptomatic, or untreated Central Nervous System (CNS) disease/metastases.
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5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** * **Investigational Arm:** Giredestrant (30 mg PO once daily) + Letrozole placebo PO once daily + Palbociclib. * **Control Arm:** Letrozole (Trade Name: Femara; ATC Code: L02BG04) (2.5 mg PO once daily) + Giredestrant placebo PO once daily + Palbociclib. **Route of Administration:** Oral (PO) **Duration of Treatment:** Until disease progression, unacceptable toxicity, or patient withdrawal.
- 5.2 Concomitant & Prohibited Therapy Permitted:** LHRH agonists (for pre/peri-menopausal women). **Prohibited:** Other systemic anti-cancer therapies, concurrent use of other SERDs, or unapproved investigational agents.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging (RECIST v1.1), Labs
Baseline	Day 0	Randomization (1:1), First Dose of Study Drug
Interim	Q4W / Q8W	Safety Labs, Efficacy Assessment (Tumor Imaging), QoL Questionnaires
End of Study	Follow-up	Final Vital Signs, Exit Interview, Disease Progression confirmation, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS), as determined by the investigator, defined as the time from randomization to the first occurrence of disease progression or death from any cause. **Measurement Method:** Investigator-assessed tumor evaluations per Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1.

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) * Objective Response Rate (ORR), as determined by the investigator according to RECIST v1.1 * Duration of Response (DOR), as determined by the investigator according to RECIST v1.1 * Clinical Benefit Rate (CBR) * Deterioration in pain level and physical functioning (QoL)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via physical examinations, vital signs, ECGs, and clinical laboratory tests; AE severity graded per NCI CTCAE. **Serious Adverse Events (SAEs):** Reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent external Data Monitoring Committee to evaluate safety and efficacy milestones.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary and secondary efficacy analyses (PFS, OS, ORR). Safety Set for participants receiving at least one dose of study drug. **Sample Size Justification:** The study targeted 992 enrolled participants to be sufficiently powered to detect a statistically significant improvement in PFS for the giredestrant + palbociclib arm versus the letrozole + palbociclib arm. **Handling of Missing Data:** Censoring rules applied for progression-free survival and overall survival as per standard FDA/EMA oncology guidelines.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits to verify data source documents, drug accountability, and protocol compliance. **Audit Trail:** Electronic Data Capture (eDC) system with full audit trail capabilities for eCRF locking and data clarification.

11. Study Identifiers & Governance

Primary ID: B041843 **Registry Number:** NCT04546009 **Posting ID:** 740358318437416861 **Sponsor/Lead Organization:** Hoffmann-La Roche
Study Title: persevERA Breast Cancer **Official Regulatory Title:** A Phase III Randomized, Double-Blind, Placebo-Controlled, Multicenter Study Evaluating the Efficacy and Safety of Giredestrant Combined With Palbociclib Compared With Letrozole Combined With Palbociclib in Participants With Estrogen Receptor-Positive, HER2-Negative Locally Advanced or Metastatic Breast Cancer

12. Clinical Framework (Breast Cancer Specific)

Primary Condition: Estrogen Receptor-Positive, HER2-Negative Locally Advanced or Metastatic Breast Cancer **Preferred Disease Name:** Malignant neoplasm of breast **Diagnosis Threshold:** Histologically or cytologically confirmed ER+/HER2- breast cancer. **Medical Methodology:** Endocrine therapy combined with a CDK4/6 inhibitor (first-line setting). **Optimization Target:** Disease control / Delaying disease progression (PFS extension).

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for advanced breast cancer, with tumor imaging at regular specified intervals. **Education Protocol (HCP):** Site Initiation Visits (SIV) and Investigator Meetings to ensure protocol and RECIST v1.1 training. **Education Protocol (Patient):** Informed consent process detailing drug interactions, dosing diary management, and reporting of adverse events. **Quality Audit Mechanism:** Centralized independent review of imaging (if applicable) and standard sponsor compliance audits.

14. Observation & Follow-Up Schedule

Total Duration: Estimated up to 90 months (from first patient in to estimated study completion in March 2028). **Onsite Visit Cadence:** Coordinated with standard 28-day treatment cycles for CDK4/6 inhibitors + endocrine therapy. **Remote Monitoring Frequency:** As dictated by the Clinical Monitoring Plan. **Checklist Review Interval:** Regular interval tumor assessments (e.g., every 8-12 weeks) until disease progression.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				

Clinical Operations Lead | [Name] | ____ | [DD-MMM-YYYY] | | Lead
Statistician | [Name] | _____ | [DD-MMM-YYYY] |